



# Fiona Stanley Fremantle Hospitals Group

## Policy and Procedure

# Discharge planning for mental health inpatients

Reference #:FSFH-MENH-POL-0032

## Scope

| Site                                         | Service/Department/Unit | Disciplines                                        |
|----------------------------------------------|-------------------------|----------------------------------------------------|
| Fiona Stanley Hospital<br>Fremantle Hospital | Mental Health Service   | Medical, Nursing, Allied Health, Pharmacy and HIMS |

## 1. Introduction

This document outlines the discharge planning requirements for patients admitted to a mental health inpatient ward. It aims to support structured discharge planning for patients and their family/significant others and smooth the transition to outpatient care by coordinating various services.

## 2. Terminology

|                                     |                                                                                                                                                                                                                                                                                                                                                                                                                                                |
|-------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Discharge planning</b>           | The development of a personalised discharge plan to ensure a patient moves smoothly from the hospital inpatient setting to wherever they are going next – it might be home, residential care, respite or somewhere else where governance of care is transferred to another service.                                                                                                                                                            |
| <b>Discharge summary</b>            | A summary of what happened during a patient's hospital stay. It must include diagnosis, outcomes of investigations, changes to treatment, referrals to services and medication changes and reason why and identifies the actions that need to be taken after discharge. It includes input from the multidisciplinary team. Used to formally communicate information to the patients General Practitioner (GP) and/or other Service Provider/s. |
| <b>Multidisciplinary Team (MDT)</b> | Refers to the treating team including psychiatrist, doctors, nursing, allied health professionals and other support staff including Peer Workers and Aboriginal Mental Health Liaison.                                                                                                                                                                                                                                                         |
| <b>Person centred care</b>          | Is respectful of, and responsive to, the preferences, needs and values of the individual patient. Involves seeking out and understanding what is important to the patient, fostering trust,                                                                                                                                                                                                                                                    |

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|-------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|                                                 | establishing mutual respect and working together to share decisions and plan care. Literature suggests that patient centred approaches reduce readmissions and increase patient satisfaction <sup>2</sup> .                                                                                                                                                            |
| <b>Transfer of care</b>                         | When all or part of a person's health care is transferred between care providers. This may occur within a service (e.g. from inpatient to community) or between different services (e.g. mental health service to a GP). It may involve transfer of responsibility for some aspects or all of a person's health care, on a temporary or permanent basis <sup>3</sup> . |
| <b>Treatment Support and Discharge Plan</b>     | Form providing a framework for summarising the goals and clinical issues to be targeted in the episode of care, with the intent of aiding the monitoring of clinical status by the patient, their family/significant other or support persons, their GP and other service providers.                                                                                   |
| <b>Patient initiated early discharge (PIED)</b> | Patient initiated early discharge which is considered by the medical officer to be safe                                                                                                                                                                                                                                                                                |
| <b>Discharge against medical advice (DAMA)</b>  | Patient initiated early discharge against the advice of the Consultant Psychiatrist and is generally considered unsafe.                                                                                                                                                                                                                                                |

### 3. Policy

#### 3.1. Key Principles

- The period immediately following discharge from hospital is a pivotal period of transition for people with mental illness<sup>1</sup>. Poor transition planning at discharge from hospital may be associated with an increased risk of decline in mental health resulting in readmission, treatment nonadherence, homelessness and suicide<sup>1</sup>.
- The mental health service recognises that the length of an inpatient admission (ranging from several hours to weeks/months) will impact the ability to effectively discharge plan however this document includes the key elements to be considered when possible.
- Discharge from a mental health inpatient setting is a significant transition of care for the patient. Safe and high-quality transitions of care<sup>3</sup>:
  - are person-centred.
  - include multidisciplinary collaboration.
  - are communicated, documented and information is accessible.
  - ensure ongoing continuity of care.
- Discharge planning will commence at the beginning of the episode of care i.e on admission to the ward or as early as possible.

- The estimated date of discharge (EDD) will be co-decided by the MDT and patient and family/significant other where possible and documented as a goal to work towards<sup>2</sup>.
- The patients EDD must be documented within 24 hours of admission and be reviewed at each MDT meeting. Refer to SMHS Discharge policy.
- Where a patient is to be followed up by a FSFHG community treatment team referrals must be directed to the respective community team at the earliest opportunity to allow for care coordinator allocation and in reach.
- The receiving community team will assess and where accepted meet the patient on the ward prior to discharge.
- Communication and planning between the treating team and receiving teams must be active and collaborative to enhance effective discharge planning<sup>4</sup>.
- The referring service will retain responsibility for the patient until handover is completed to the receiving service or practitioner, or the patient decides on an alternative process.
- The patients General Practitioner (GP) will be provided with a discharge summary within 24 hours of discharge to prevent treatment delays and minimise the risk of adverse events due to missing information.

## 4. Procedure

- Discharge planning will:
  - be trauma informed and recovery-oriented, with staff making every effort to support the patient's rights, choice and self-determination<sup>4</sup>.
  - consider the patient's language, culture and diversity, gender and/or sexual orientation.
  - involve the patient's family/significant others and relevant stakeholders and/or support agencies where appropriate.
  - involve Aboriginal Mental Health Liaison (AMHL) staff and/or significant members of the patient's community including Elders and Traditional Healers.
  - include Peer Workers where appropriate.
  - utilise interpreter services and alternative modes of communication for patients who have communication barriers.

### 4.1. Partnering with family/significant others

- On admission ask the patient to identify their family/significant other or advocate.

- Document details on the Mental Health Consent for Sharing of Consumer Information form.
- Inform the family/significant other of the key contact in the MDT for discharge planning and provide a copy of the Carers Welcome letter (Inpatient).
- Communication and planning between the treating team and family/significant other will be active and collaborative.
- Consult with Social Work if a family/significant other is an identified or suspected person using violence, especially where there are children. Safety and risk to children and victim survivor are the priority. Refer to the FSFHG Family and Domestic Violence Policy.
- All Aboriginal or Torres Strait Islander patients must have an eReferral forwarded to the AMHL on admission to ensure cultural safety and security:
  - The AMHL will provide an initial meet and greet with the patient providing information around the AMHL service and supports provided.
  - The patient has the right to accept or decline further AMHL input in their recovery, treatment support and discharge planning.
- The treating team will:
  - advise the family/significant other of likely discharge date well in advance and involve them in treatment and discharge planning from an early stage and throughout the inpatient admission<sup>1</sup>.
  - involve family /significant other in developing the Treatment Support and Discharge Plan (TSDP) with the patient's permission.
  - arrange family /discharge planning meetings as required to identify issues/concerns and discuss identified risks. Update family/significant other regarding care planning and discharge processes.
  - discuss with family/significant others the patients progress and discharge readiness and safety planning where appropriate.
- The Treating Team will consider the appropriate psychoeducation support required for family/significant others to improve long term outcomes for the patient including reducing relapse and readmission<sup>1</sup>. This could include:
  - signposting to specific condition information
  - local carer support services.

#### 4.2. Working with other services

- On admission the treating team will identify the wider team of key stakeholders to be involved in the patients discharge plan. This may include GP, Office of the Public Advocate, NDIS, Aged Care, Corrective Services, Alcohol and Other Drug services, Family and Domestic Violence Services, schools, Department of Communities and non-government organisations.

- Discuss with the patient whether they consent to share information with key stakeholders.
- Document consent on the MH Consent for sharing of consumer information form or the NDIS Consent form as appropriate.
- Engage key stakeholders in discharge planning at key points throughout the admission. This could be via:
  - Telephone
  - Telehealth
  - MS Teams
  - Designated key stakeholder meetings
  - Discharge planning meetings.
- Define and communicate the roles, responsibilities, and expectations of the treating team and key stakeholders involved in the patient's care early in the admission to enhance collaborative working, improve outcomes for consumers and reduce unnecessary delays to discharge.
- Assist the patient to maintain community links during their inpatient admission<sup>1</sup>. This may include phased return to their employment or education setting and/or continuing appointments in the community e.g. community/social access support from NDIS, attending outpatient group therapy.
- Where possible before discharging a school aged patient a meeting will be arranged between a clinician, the patient and a named person from the education setting to plan their return to learning<sup>5</sup>.
- Where a patient is being discharged to a psychiatric hostel, supported accommodation or residential aged care facility involve the relevant managers, clinicians and support staff in treatment and discharge planning.
- For patients requiring care supports on discharge, the treating team will inform the relevant agencies/stakeholders of the patients EDD in a timely manner to allow adequate time for supports to be arranged and set-up.
- Refer the patient to the NDIS Hospital Liaison Officer (HLO) if the MDT determines access to disability supports (i.e.. NDIS) is essential for a safe and sustainable discharge, and the patient consents. Commence on the hospital pathway to expedite this process.

### 4.3. Accommodation

- Review and confirm the patient's current housing arrangements early in the admission.

- Refer patients who are homeless or are at risk of homelessness and require support finding accommodation to the Welfare Officer and/or Social Worker for assistance to facilitate safe and sustainable discharge.
- The service will explore all housing options with the patient including crisis, short term, long-term and transitional care, managing expectations directly with the patient and their families/significant others. Patients will be requested to engage in exploring housing where they have capacity to do so.
- Patients at significant risk e.g. domestic violence should not be discharged to unreasonable accommodation or homelessness.
- Patients who are homeless will be discharged into appropriate accommodation and/or referred to local homelessness services<sup>4</sup>.
- Patients who are unable to be discharged into accommodation will be followed up as per Section 4.8 Follow up care.

#### 4.4. Preparing for discharge

- The treating team will:
  - conduct a thorough assessment of the patients personal, social, safety and practical needs to support discharge<sup>1</sup>.
  - the assessment must include risk of harm to self and/or others and from others.
  - arrange a discharge planning meeting including the multiple stakeholders involved in the patient's overall care/treatment where relevant.
- The MDT will ensure the AMHL is present at patient reviews (where the patient has consented to AMHL input) to provide relevant cultural input and support in treatment and discharge planning, tribunal hearings, family meetings and other disciplined based assessments.
- Graduated leave will be supported for the patient prior to discharge where possible. Approved leave from the inpatient ward is an important part of discharge preparation and allows the patient and the treating team to assess readiness for discharge to the community<sup>4</sup>.
- Planning for discharge will:
  - include appropriate safety planning.
  - involve the patient as fully as possible and be developed collaboratively with family/significant others where possible.
  - relate directly to the setting the patient is being discharged to e.g. home, supported accommodation, residential aged care facility.
  - consider the individualised psychoeducation sessions and harm reduction required to promote learning and awareness for managing the patient's condition.

- include medication review identifying medications that have been added, discontinued, or changed<sup>6</sup>.
- ensure the patient and family/significant other understands what medications they are taking, how to take them and why they are taking them<sup>6</sup>.
- cover aspects of the patient's daily life e.g. social activities, physical health
- explore financial situation and transport needs including in relation to being able to attend appointments for follow up.
- include access to necessary services and referrals for ongoing care and supports.
- communication and liaison as appropriate with new, existing and former service providers
- ensure access to necessary patient equipment (i.e loan equipment from Patient Equipment Centre)
- The medical team is responsible for writing discharge prescriptions and for notifying the clinical pharmacist in advanced of discharge.
- Prescriptions must be forwarded to the pharmacy by 15:00hrs the day prior to discharge (before midday on the weekend) to:
  - allow timely delivery.
  - minimise delays.
  - ensure timely clinical review of prescriptions by the ward pharmacist.
  - ensure timely dispensing by Pharmacy.
  - ensure the patient and/or carer receives appropriate medication counselling and written medication information.
- Where a patient requires a dose administration aid (DAA) e.g. Webster Pak or sachets, prescriptions must be provided to the clinical pharmacist at least 24 hrs prior to the expected discharge time.
- Written and/or verbal medication education must be provided to the patient/family significant other when there is a change to ongoing therapy and/or prior to discharge e.g.
  - The doctor will discuss any medication changes with the patient, including potential interactions with substance use.
  - The Pharmacist will add specific medication information as appropriate including details such as clarifying prescription purpose, administration instructions, side effects etc.

### 4.5. Discharge clinical communication.

- At the time of discharge patients will have the following completed:



- Risk Assessment and Management Plan (RAMP)
- National Outcome and Case-mix collection (NOCC)
- A copy of the current Treatment Support and Discharge Plan provided to everyone involved in supporting the patient at discharge and afterwards<sup>5</sup>.
- Inpatient Discharge Planning Checklist
- All patients will have a discharge summary completed in Notification and Clinical Summaries (NaCS) within 24 hours of discharge and forwarded to the patients GP. The Discharge summary will include:
  - episode of care summary
  - diagnostic issues and formulation
  - risk assessment and management
  - signs of relapse and coping strategies with a dual diagnosis focus where appropriate.
  - recovery goals
  - current medication list
  - follow up required including physical health checks and monitoring.
- The referring team will provide the receiving team/service with verbal clinical handover of all relevant information for safe transfer of care using iSoBAR where possible.
- Key stakeholders providing ongoing support for the patient will receive written and verbal discharge information from the inpatient treating team.

#### 4.6. Patient education and discharge information

- The patients written discharge plan will be reviewed with the patient and family / significant other prior to discharge (unless the treating psychiatrist reasonably believes that is not in the patient's best interests) to ensure understanding.
- The discharge plan and patient education must be trauma informed, clear and in a format the patient can understand<sup>6</sup>. Use appropriate illustrations to explain concepts. Consider who else may be reading the discharge plan.
- Focus on information critical to the patient and their family/significant others understanding of how to manage their condition after discharge<sup>6</sup>. Patients identified as at risk of suicide will also have a safety plan documented.
- Discharge plans will at a minimum include:
  - where relevant the time and date of upcoming outpatient appointments
  - details of medications prescribed on discharge.



- name and contact details of community team (if referred) and the name of the Care Coordinator
- support arrangements after discharge.
- signs of relapse and self-monitoring strategies
- contact details for emergency services e.g. Mental Health Emergency Response Line, local Emergency Department
- information on how to re-enter the service.

#### 4.7. Follow up care.

- An attempt will be made to make direct contact as soon as possible after discharge, and at least within 24-48 hours with consumers who may be at risk of suicide or as agreed in the consumer's safety plan.
- Discharge follow up of mental health inpatients will be in line with the FSFHG Discharge follow up for mental health inpatients policy.
- An outpatient psychiatric appointment will be scheduled within 7 days of discharge where possible. Scheduling an outpatient appointment as part of a patient's discharge plan is significantly associated with the patient attending outpatient appointments<sup>6</sup>. The patient will be provided clear plans for next actions/follow up at the post inpatient discharge appointment.
- Any clinical deterioration identified during follow up must be escalated and managed appropriately.

## 5. Unplanned discharge

- If a patient is likely to or could be discharged after hours, the Consultant psychiatrist and treating team will ensure that instructions for discharge are available. The instructions will include at a minimum:
  - Follow up required e.g. referral to Assessment and Treatment Team, WAPOL, CTT, Child Protection, Non-Government Organisations
  - Risk assessment
  - Safety planning
  - Medication requirements

#### 5.1. Patient initiated early discharge.

- PIED of voluntary patients will be determined by the:
  - In hours the MDT and Consultant Psychiatrist. Refer to the FSFHG Patient Leave and Escort - Mental Health policy.

- After hours the Duty Medical Officer (DMO)/After Hours Registrar. They must contact the on-call Consultant Psychiatrist to discuss the proposed discharge.
- PIED of voluntary patients must be communicated to the family/significant other and key stakeholders and discharge instructions relayed.

## 5.2. Discharge against medical advice

- DAMA of voluntary patients must be discussed with:
  - In hours the MDT and Consultant Psychiatrist to determine required actions may include attempting to contact the patient, notifying family/significant other and key stakeholders. An appropriate escalation plan is to be developed in relation to acuity and risk. Refer to the FSFHG Patient Leave and Escort - Mental Health policy.
  - After hours the Duty Medical Officer (DMO)/After Hours Registrar will be requested to review any voluntary patient who seeks to discharge themselves or has DAMA. They must contact the on-call Consultant Psychiatrist to discuss the proposed discharge before it occurs where possible. Refer to the FSFHG Patient Leave and Escort - Mental Health policy.

## 5.3. Absent without leave.

- Involuntary patients under the Mental Health Act 2014 who are Absent Without Leave (AWOL) may be discharged from the inpatient setting if the Consultant Psychiatrist deems it appropriate to revoke their current legal form and either place them on a Community Treatment Order or change their MHA 2014 status to voluntary.
- An involuntary patient's family/significant other must be informed when they are AWOL, leave is varied/cancelled, or patient is discharged.
- If the Consultant Psychiatrist deems unplanned discharge is inappropriate or unsafe relevant actions must be taken according FSFHG Patient Leave and Escort – Mental Health policy.

# 6. Compliance/Performance Monitoring

Service 5 will monitor compliance with this policy through monthly Health Service Performance Reporting Indicators:

- P1-1: Percentage of post discharge community care within 7 days following discharge from acute specialised mental health inpatient services.
- P2-21: Readmissions to acute specialised mental health inpatient services within 28 days of discharge

- P2-25 Percentage of inpatient discharge summaries completed within 24 hours.

## 7. Related Policy Documents

DoH:

- [Safety Planning for Mental Health Consumers Policy MP 0181/24](#)
- [Safety Planning Procedures for Mental Health Consumers MP 0181/24](#)
- [Statewide Standardised Clinical Documentation for Mental Health Services MP 0155/21](#)
- ['Triage to Discharge' Mental Health Framework for Statewide Standardised Clinical Documentation](#)

SMHS

- [Discharge \(SMHS: 8\)](#)
- [Discharge summary \(SMHS: 7\)](#)
- [Discharge against medical advice \(SMHS: 4\)](#)
- [Missing or suspected missing mental health patients \(SMHS: 47\)](#)

FSFHG:

- Discharge: FSFHG Bed Management (FSFH-HW-POL-0004)
- Discharge follow up for mental health inpatients (FSH-MENH-POL-0019)
- Patient leave and escort – Mental Health (FSFH-MENH-POL-0022)
- Inpatient admission to discharge guidelines and responsibilities (FSFH-MENH-GUI-0011).
- Medication reconciliation on admission, transfer and discharge (FSFH-HW-POL-0021)
- Medication review (FSFH-HW-PRO-0014)
- Clinical documentation (FSFH-HW-POL-0039)
- Clinical handover (FSFH-HW-POL-0035)
- Patient initiated early discharge (FSFH-HW-GUI-0039)
- Family and Domestic Violence (FSH-HW-PRO-0007)
- Assessment and Treatment Team: referral for out of hours follow up (FSFH-MENH-PRO-0011)
- Did not attend: outpatient appointments (FSFH-MENH-PRO-0001).
- Clinical Risk: Assessment and Management (FSFH-HW-POL-0017)

## 8. Related documents

The Mental Health Act 2014

## 9. Related Standards

NSQHS standards:

- Partnering with consumers
- Medication safety
- Comprehensive care
- Communicating for safety
- Recognising and responding to acute deterioration

Chief Psychiatrists Standards for Clinical Care

- Assessment
- Care planning
- Consumer and carer engagement
- Transfer of care

## 10. References

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3. Australian Commission on Safety and Quality in Health Care. Fact sheet - Principles of safe and high-quality transitions of care. 2024. Retrieved from <https://www.safetyandquality.gov.au/publications-and-resources/resource-library/fact-sheet-principles-safe-and-high-quality-transitions-care>
4. NSW Ministry of Health. (2019) Policy Directive PD2019\_045. Discharge planning and transfer of care for consumers of NSW Health Mental Health Services. Retrieved from [https://www1.health.nsw.gov.au/pds/ActivePDSDocuments/PD2019\\_045.pdf](https://www1.health.nsw.gov.au/pds/ActivePDSDocuments/PD2019_045.pdf)
5. National Institute for Health and Care Excellence (NICE). (2016, August).

Transition between inpatient mental health settings and community or care home settings - NICE guideline [NG53]. Retrieved from <http://www.nice.org.uk/guidance/ng53>

6. Alper, E. O'Malley, TA. And Greenwald, J. (2022). Hospital Discharge and Readmission. Retrieved from <http://www.uptodate.com>.

## 11. Authorisation

| EXECUTIVE SPONSOR: Service Director, Service 5 |             |                           |                                    |                                             |              |
|------------------------------------------------|-------------|---------------------------|------------------------------------|---------------------------------------------|--------------|
| Version                                        | Date Issued | Compiled/Revised By       | Committee/Consumer Group Consulted | Endorsed By                                 | Revision due |
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